INFORMATION TO KEEP WITH YOU

My complete treatment handbook

AC → weekly paclitaxel + weekly trastuzumab

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
Intervals differ by treatment phase

Treatment stages
3
Explained in the treatment reference

Regimen source
NCCP 00432
Version 5

Four AC cycles 21 days apart, then 12 weekly paclitaxel doses with weekly trastuzumab. Continue trastuzumab to complete one year total, without overlapping it with AC.

Treatment stages

01 **Phase 1 · AC**
21-day interval

02 **Phase 2 · Paclitaxel with HER2 treatment**
7-day interval

03 **Review / HER2 continuation**
21-day interval

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new breathing symptoms, severe diarrhoea, inability to drink, painful swollen limbs, unusual bleeding, or worsening numbness. Tell staff immediately about infusion-site pain or an allergic reaction.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

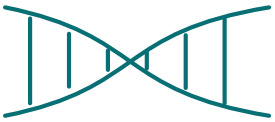
This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

How the medicines work

Part of an individual breast cancer plan to reduce the risk of recurrence, and to shrink the cancer before surgery where that is the agreed setting.

ANTHRACYCLINE CHEMOTHERAPY

Doxorubicin



Interferes with deoxyribonucleic acid (the cell's genetic material) (DNA) handling

Doxorubicin interferes with enzymes and DNA processes needed for cell division. It can also affect heart muscle, so the team considers previous exposure and cardiac risk.

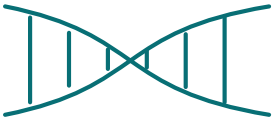
DNA handling disrupted → Cell damage → Division reduced

What to watch for

Report pain or burning at the infusion site immediately. New breathlessness or chest symptoms need prompt review. Red urine for a short time can be a drug-colour effect; pain, clots or persistent bleeding still need advice.

ALKYLATING CHEMOTHERAPY

Cyclophosphamide



Damages DNA needed for growth

Cyclophosphamide is activated in the body and damages DNA. This helps stop dividing cancer cells but can also affect blood-cell production and reproductive tissues.

Drug activated → DNA damage → Cell division affected

What to watch for

Use the prescribed anti-sickness and hydration plan. Report fever, painful urination or blood in the urine.

TAXANE CHEMOTHERAPY

Paclitaxel

Taxol (conventional paclitaxel)



Locks the cell division machinery

Cells use tiny structures called microtubules to separate their chromosomes when dividing. Paclitaxel stabilises these structures so they cannot rearrange normally, interrupting cell division. It can also affect nerves and other healthy tissues.

Microtubules form → Structures remain locked → Division is interrupted

What to watch for

Report tingling or numbness before the next next dose, especially if it affects walking or hand use. Tell the nurse immediately about flushing, chest or throat tightness, itching, dizziness or breathing changes during infusion.

HER2-TARGETED ANTIBODY

Trastuzumab



Targets HER2 growth signals

Trastuzumab binds to the HER2 protein on cells and helps reduce HER2-driven growth and recruit immune activity. HER2 testing identifies the cancers for which this approach is used.

HER2 on cell surface → Antibody binds → Growth signalling reduced

What to watch for

Keep echo appointments throughout HER2 treatment. Report breathlessness, swelling, palpitations or a reaction; chemotherapy side effects are tracked separately.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment?

This sequence combines chemotherapy with trastuzumab because the cancer is HER2-positive. AC comes first; trastuzumab starts with paclitaxel and continues after chemotherapy. Receptor testing, recurrence risk, heart health and other HER2 options determine whether this is the right sequence.

Your treatment setting

HER2-positive neoadjuvant or adjuvant high-risk breast cancer

The sequence

Four AC cycles 21 days apart, then 12 weekly paclitaxel doses with weekly trastuzumab. Continue trastuzumab to complete one year total, without overlapping it with AC.

Alternatives and your priorities

Ask what alternatives fit your cancer, what the treatment aims to achieve, and how its practical demands fit your life. The team can discuss personal benefits and risks.

Why this treatment may be chosen

Show trial results and what the numbers mean

- Pembrolizumab + chemotherapy
- Placebo + chemotherapy

Your team can discuss expected benefit and alternatives in the context of your own cancer and priorities.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

The phases and how they connect

Treatment stages

01 **Phase 1 · AC**
21-day interval

02 **Phase 2 · Paclitaxel with HER2 treatment**
7-day interval

03 **Review / HER2 continuation**
21-day interval

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

21-DAY INTERVAL

Phase 1 · AC

4 cycles

Four AC cycles 21 days apart. Growth-factor use follows the regimen and individual risk plan.

7-DAY INTERVAL

Phase 2 · Paclitaxel with HER2 treatment

12 weekly taxane doses

Four AC cycles 21 days apart, then 12 weekly paclitaxel doses with weekly trastuzumab. Continue trastuzumab to complete one year total, without overlapping it with AC.

21-DAY INTERVAL

Review / HER2 continuation

Individualised after review

Continue the source 21-day HER2 schedule after chemotherapy to complete one total year, unless clinical findings change treatment.

The treatment phase determines the medicines, supportive prescription and tests. A phase change does not reset previous drug exposure or cardiac monitoring.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. The clinical team owns booking, result review and action on overdue tests. This is an educational map, not an automated appointment or result tracker. EMA supplementary follow-up: cardiac assessment every 6 months after trastuzumab until 24 months; after anthracyclines in early disease, yearly to5 years from the last trastuzumab dose, or longer if function falls.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00432 v5 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

AC → weekly paclitaxel + weekly trastuzumab: treatment and checks

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early cycles	Later cycles / transition	Next treatment / completion	Follow-up
Cancer medicines NCCP 00432	Agree aim, indication and medicines	AC → weekly paclitaxel + weekly trastuzumab starts	Change from AC to paclitaxel; select new support script	Stop paclitaxel → trastuzumab alone	Complete one total year of trastuzumab
Surgery EARLY	Adjuvant: surgery already completed	Neoadjuvant: chemotherapy first	Review response / surgical plan	Surgery after neoadjuvant therapy, if applicable	Pathology and recovery guide next steps
Radiotherapy EARLY	Assess indication	Usually after cytotoxic chemotherapy	Arrange sequential timing	Radiotherapy if indicated; HER2 may overlap by multidisciplinary team (MDT) plan	Review effects / follow-up
Blood tests NCCP 00432	full blood count (FBC), kidney and liver profile	Review before chemotherapy cycles	Check again at any phase change	Follow active medicines and abnormalities	Follow unresolved abnormalities
Echo / heart NCCP 00432 · ESC	Baseline before AC	Reassess before starting trastuzumab	Every 12 weeks during trastuzumab	Continue the same12-week clock after paclitaxel	After trastuzumab: follow EMA/ ESC and cardiac risk plan
electrocardiogram (ECG) NCCP 00432	Baseline ECG in the NCCP anthracycline source; repeat for symptoms or clinical risk. An ECG does not replace an echo.	Symptoms / risk-based repeat	Review if cardiac concern	ECG and echo answer different questions	Clinical indication

Care / check	Before starting	Early cycles	Later cycles / transition	Next treatment / completion	Follow-up
Cancer imaging NCCP 00432	Record baseline extent as appropriate	Clinical response / symptoms review	If before surgery: plan response review	Arrange selected surgery / next treatment	Planned breast imaging; no routine computed tomography (CT)/positron emission tomography (PET)
Markers / bone health NCCP 00432	Tests selected for active medicines	No routine tumour-marker clearance	No routine blood tumour markers when well	dual-energy X-ray absorptiometry (bone density) (DEXA) if endocrine/ bone-risk indication	Follow the endocrine and bone-health plan
Endocrine / bone BONE · EARLY	Confirm hormone receptors	Plan endocrine therapy when appropriate	Separate from this chemotherapy prescription	Start/continue per MDT; DEXA when indicated	Risk-based bone review

What each check is for

1 Before first dose

Baseline

Baseline FBC, kidney and liver tests; review before each AC cycle and each paclitaxel administration under the regimen and local testing plan.

Ask / act: Confirm tests and consent before prescribing.

2 Heart-monitoring plan

Baseline → cycles → after treatment

ESC supplementary guidance: baseline echo before anthracycline treatment. High/very-high cardiac risk: echo every two anthracycline cycles (for example before cycles 3 and 5 when continuing), and within 3 months after completion. All adults: echo within 12 months after completion. Other repeat tests depend on risk, cumulative exposure and symptoms. Reassess before the first trastuzumab, then echo/multigated acquisition (heart pumping) scan (MUGA) every 12 weeks during trastuzumab. In an uninterrupted weekly taxane phase,12 weeks is near its end; in21-day continuation this is about every 4 cycles. Keep the original clock through the transition. Earlier assessment is needed for concern.

Ask / act: Name the team responsible for booking and reviewing results.

3

Treatment-phase review

Before each transition

Four AC cycles 21 days apart, then 12 weekly paclitaxel doses with weekly trastuzumab. Continue trastuzumab to complete one year total, without overlapping it with AC.

Ask / **act:** Confirm active medicines, support script and the next test checkpoint.

4

Response and follow-up

Planned review points

For early breast cancer, breast imaging and clinical review follow the agreed pathway. Routine CT/PET scans and blood tumour markers are not used to look for distant recurrence when there are no symptoms or clinical concerns.

Ask / **act:** Review symptoms and imaging together; do not change treatment on tumour markers alone.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan

STEP 1

Before the first visit

Blood tests, relevant cardiac assessment, medicines review, fertility discussion and the consent conversation come first.

STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

Staff check the line, give prescribed premedicines and treatment, and observe for reactions. Tell them immediately about burning, pain, rash or breathlessness.

STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit time and transport

First visits can take longer because of loading doses, premedicines and observation. Ask the unit about expected time, transport and any escort needed after sedation or line insertion.

Phase-specific visit pattern

Weekly taxane visits become less frequent if HER2 continuation switches to every three weeks. Infusion and observation times differ by drug and first versus later doses; the unit confirms the full visit duration, including blood review and preparation.

Checks across treatment

Your team arranges, reviews and acts on tests; an overdue result needs follow-up.

Blood tests

Baseline FBC, kidney and liver tests; review before each AC cycle and each paclitaxel administration under the regimen and local testing plan.

Heart monitoring

ESC supplementary guidance: baseline echo before anthracycline treatment. High/very-high cardiac risk: echo every two anthracycline cycles (for example before cycles 3 and 5 when continuing), and within 3 months after completion. All adults: echo within 12 months after completion. Other repeat tests depend on risk, cumulative exposure and symptoms. Reassess before the first trastuzumab, then echo/MUGA every 12 weeks during trastuzumab. In an uninterrupted weekly taxane phase,12 weeks is near its end; in21-day continuation this is about every 4 cycles. Keep the original clock through the transition. Earlier assessment is needed for concern.

ECG

Baseline ECG in the NCCP anthracycline source; repeat for symptoms or clinical risk. An ECG does not replace an echo.

Cancer scans

For early breast cancer, breast imaging and clinical review follow the agreed pathway. Routine CT/PET scans and blood tumour markers are not used to look for distant recurrence when there are no symptoms or clinical concerns.

Surgery, radiotherapy and later treatment

Adjuvant pathway: surgery → chemotherapy → radiotherapy if indicated. Neoadjuvant pathway, where this regimen is indicated: chemotherapy → surgery → selected postoperative treatment and radiotherapy. Lumpectomy versus mastectomy is a surgical/MDT decision. Trastuzumab continuation may overlap radiotherapy under the MDT plan; it must not overlap the AC phase.

Hormone-receptor-positive disease

Endocrine treatment is considered when appropriate for the cancer biology and setting. It has its own schedule and monitoring; it is not automatically part of this chemotherapy prescription.

Bone health

DEXA is linked to an aromatase inhibitor, ovarian suppression, treatment-induced menopause or other bone risk—not automatically to every chemotherapy course. The endocrine team confirms baseline testing and risk-based repeat scans.

Markers and follow-up

After local treatment, the team plans mammography of remaining breast tissue. A mastectomy side does not need routine mammography. Report new symptoms rather than waiting for a scan.

Genomic tests

Where appropriate, the MDT may discuss a validated genomic test to inform the chemotherapy decision. This regimen card does not calculate an Oncotype result or select treatment from a score.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

AC · 21-day reference cycle

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

AC · 21-day reference cycle

Treatment or check	Timing in this view
Treatment · Day 1	 **Day 1** intravenous (IV)
Administration reaction	 **Days 1–21** Report during or after any visit
Nausea / appetite	 **Days 1–5** May occur early or later
Blood counts / infection	 **Days 1–21** Risk varies; fever needs prompt advice
Fatigue / mouth changes	 **Days 1–21** Variable timing and severity

No dates to enter: The chart explains the pattern. Your team books actual appointments and reviews results.

Phase changes: Supportive medicines and symptom expectations change when chemotherapy stops. HER2-only treatment does not automatically inherit chemotherapy premedicines.

Treatment delays: A delayed dose does not automatically delay an echo or other test that is already due.

Why these bands stay broad: These broad symptom bands are educational possibilities, not measured risk curves. Infusion days are the planned schedule; delays require a new clinical plan. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00432 v5 · official regimen ↗

There are no guaranteed “safe days”. Fever, shivers, sudden illness, severe breathing changes or chest pain need prompt assessment on any day.

Phase 2 · Paclitaxel with HER2 treatment

Day 1 → Day 7: a general pattern

Phase 2 · Paclitaxel with HER2 treatment

Treatment or check	Timing in this view
HER2 medicine · Day 1	 **Day 1** IV
Chemotherapy · Day 1	 **Day 1** IV
Administration reaction	 **Days 1–7** Report during or after any visit
Nausea / appetite	 **Days 1–5** May occur early or later
Blood counts / infection	 **Days 1–7** Risk varies; fever needs prompt advice
Fatigue / mouth changes	 **Days 1–7** Variable timing and severity
Nerves / skin / nails	 **Days 1–7** Can build across doses
Heart / breathing symptoms	 **Days 1–7** Report on any day

No dates to enter: The chart explains the pattern. Your team books actual appointments and reviews results.

Phase changes: Supportive medicines and symptom expectations change when chemotherapy stops. HER2-only treatment does not automatically inherit chemotherapy premedicines.

Treatment delays: A delayed dose does not automatically delay an echo or other test that is already due.

> **Why these bands stay broad:** These broad symptom bands are educational possibilities, not measured risk curves. Infusion days are the planned schedule; delays require a new clinical plan. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00432 v5 · official regimen ↗

HER2-only continuation · source 21-day schedule

Day 1 → Day 21: a general pattern

HER2-only continuation · source 21-day schedule

Treatment or check	Timing in this view
Treatment · Day 1	1 — 21 **Day 1** IV
Administration reaction	1 — 21 **Days 1–21** Report during or after any visit
Heart / breathing symptoms	1 — 21 **Days 1–21** Report on any day

No dates to enter: The chart explains the pattern. Your team books actual appointments and reviews results.

Phase changes: Supportive medicines and symptom expectations change when chemotherapy stops. HER2-only treatment does not automatically inherit chemotherapy premedicines.

Treatment delays: A delayed dose does not automatically delay an echo or other test that is already due.

> **Why these bands stay broad:** These broad symptom bands are educational possibilities, not measured risk curves. Infusion days are the planned schedule; delays require a new clinical plan. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00432 v5 · official regimen ↗

Between-treatment notes

Day 1–21: Your current reference cycle

Four AC cycles 21 days apart, then 12 weekly paclitaxel doses with weekly trastuzumab. Continue trastuzumab to complete one year total, without overlapping it with AC. Follow the team's appointment and medicine instructions; contact them about missed doses or new symptoms.

Support around the medicines

1  **The correct phase prescription**

AC is high emetogenic risk; a separate low-risk taxane script replaces it.

2 **Premedicines and growth factors**

Paclitaxel premedicines differ between weekly and fortnightly dosing. Use the phase-specific prescription.

3 **Tests and appointments**

The team should name the booking owner, due window and person reviewing each required test. Tell them if an expected appointment has not arrived.

4 **After chemotherapy**

HER2-only continuation has its own minimal-emetic-risk plan; do not automatically continue chemotherapy steroids, antiemetics or growth factors.

Questions worth asking

When are heart checks due?

Baseline before AC; reassess before starting trastuzumab. Then every 12 weeks during HER2 treatment. Count 12 weeks from first HER2 treatment (after AC where applicable): around weekly antibody dose 13 or q21-day cycle 5. Preserve elapsed time across later phase / route changes and delays.

Where do surgery and radiotherapy fit?

Two possible pathways: neoadjuvant systemic treatment → surgery / pathology → postoperative plan; or surgery first → adjuvant systemic treatment → radiotherapy when indicated. HER2-only treatment, if prescribed, may overlap radiation / endocrine therapy under MDT review. These are alternatives, not two consecutive surgery courses.

Do I need routine CT scans or tumour markers?

Early breast follow-up uses planned breast imaging. Routine CT / PET, blood tumour markers or other distant-recurrence screening are not recommended without symptoms or a clinical indication. After local treatment: annual mammography of remaining breast tissue; additional imaging when indicated.

Does a port mean a pump at home?

No. This regimen has no routine take-home infusion pump. A port is an access device; the team removes the needle after the visit when appropriate and arranges device removal separately.

Who arranges the scans and tests?

Your treating service should book the planned checks, review the results and explain any change. If an expected appointment is missing, contact the team; this map does not book tests or confirm that they have happened.

Try explaining the plan back: Can you explain which medicines you are receiving now? What symptoms mean you should call urgently? Which blood tests, scans or heart checks come next, and who arranges them?

Doxorubicin

- Low counts, infection and mouth ulcers
- Nausea, hair loss and fatigue
- Heart damage, sometimes delayed
- Severe tissue injury if the infusion leaks
- Rare later blood cancers

Cyclophosphamide

- Low counts and infection
- Nausea, vomiting and hair loss
- Bladder irritation / bleeding
- Fertility effects and menopause

Paclitaxel

- Low blood counts and infection
- Numbness / tingling, sometimes lasting
- Hair loss, muscle or joint pain
- Infusion reaction
- Mouth, bowel, skin and nail effects

Trastuzumab

- Heart pumping-function reduction / heart failure
- Infusion or allergic reaction
- Diarrhoea and fatigue
- Rare serious lung effects

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

AC · 21-day schedule

Active medicines: Doxorubicin + Cyclophosphamide

Emetogenic risk: High · AC

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1
Ondansetron	16 mg orally	Day 1 Individual dose / QT / hepatic review
Dexamethasone	12 mg orally	Day 1 No routine Days 2–4 dexamethasone in this NCCP AC antiemetic row
Olanzapine	5 mg orally	Days 1–4 Review sedation, falls, interactions and glucose
Aprepitant	80 mg orally daily	Days 2–3
Metoclopramide	10 mg orally up to three times daily	Subsequent days: duration and use to be confirmed Caution with olanzapine: additive extrapyramidal effects; adjust / avoid for individual contraindications

Additional phase-specific support

- Assess granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) according to source guidance and individual risk.
- This AC antiemetic script is replaced at the taxane transition.
- At the AC-to-paclitaxel transition, stop using this AC antiemetic template and select the taxane phase. Review residual symptoms separately.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Taxane / combination phase

Active medicines: Paclitaxel + Trastuzumab

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- First paclitaxel dose: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before treatment (NCCP suggested table).
- From Day 8 onward: chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before treatment. Routine dexamethasone may be omitted after the first dose; prior reaction or increased risk requires consultant review.
- Famotidine may be omitted under the consultant/local plan if appropriate. A prior reaction requires a separate rechallenge assessment; do not reuse the standard draft automatically.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

HER2-only continuation

Active medicines: Trastuzumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Routine prophylactic antiemetics and paclitaxel premedicines are not carried into this phase. Treat prior nausea / infusion reactions under an individual plan.
- Continue HER2 cardiac monitoring; ending paclitaxel does not end echo surveillance.
- NCCP 00432 transitions after 12 weekly combination doses to trastuzumab 6 mg/kg every 21 days under regimen 00200, to complete one total year. Confirm the prescribed schedule and missed-dose rules; do not reuse weekly loading logic for 21-day continuation.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

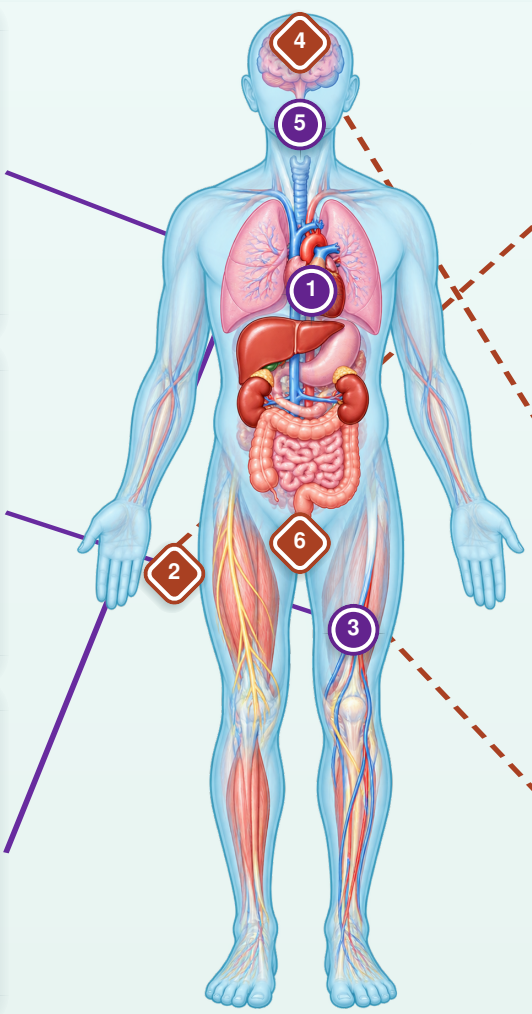
AC → weekly paclitaxel + weekly trastuzumab

Four AC cycles 21 days apart, then 12 weekly paclitaxel doses with weekly trastuzumab. Continue trastuzumab to complete one year total, without overlapping it with AC.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Heart

Doxorubicin / trastuzumab

Baseline assessment; report chest pain, breathlessness or swelling.

2 Hands / feet

Paclitaxel

Report numbness or tingling before the next dose.

3 Blood / infection

Chemotherapy phase

Fever, shivers or suddenly unwell: call immediately.

4 Hair / scalp

Chemotherapy phase

Hair loss is common. Ask about scalp cooling before treatment where suitable.

5 Mouth

Chemotherapy phase

Painful ulcers or difficulty drinking need advice.

6 Bladder

Cyclophosphamide

Report blood in urine or painful urination.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Selected medicine-specific effects are highlighted. The map is not a complete list; persistent or new symptoms need review even after a medicine stops.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **Regimen selection and source references**
   https://healthservice.hse.ie/documents/6470/432_v5_AC-TH_weekly.pdf
2. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
3. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
4. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
5. **ESMO metastatic breast cancer guideline · 2026**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
6. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
7. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
8. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
9. **NOGG · section 7, aromatase inhibitors and fracture risk**
   https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
10. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
11. **ASCO · biomarkers in metastatic breast cancer**
    https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
12. **EMA Herceptin product information**
    https://www.ema.europa.eu/en/documents/product-information/herceptin-epar-product-information_en.pdf